

Chicago Cosmetic Dermatology and Surgery, Illinois,^a and Department of Dermatology, University of California Davis Medical Center, Sacramento^b

Funding sources: None.

Conflicts of interest: None declared.

Correspondence to: Daniel B. Eisen, MD, Department of Dermatology, University of California Davis Medical Center, 3301 C St, #1400, Sacramento, CA 95816

E-mail: dbeisen@ucdavis.edu

REFERENCES

1. Campbell T, Armstrong AW, Schupp CW, Barr K, Eisen DB. Surgeon error and slide quality during Mohs micrographic surgery: is there a relationship with tumor recurrence? *J Am Acad Dermatol* 2013;69:105-11.
2. Hruza GJ. Mohs micrographic surgery local recurrence. *J Dermatol Surg Oncol* 1994;20:573-7.

<http://dx.doi.org/10.1016/j.jaad.2013.10.027>

Generational gap regarding patients' driving habits

To the Editor: The article, "Sunscreen use while driving,"¹ serves as a valuable reminder of the importance of proper patient education regarding sunscreen use. The authors found that only 26% of the patients surveyed used sunscreen at least half the time while in the car. They drove the importance home by noting that 62% of the nonmelanoma skin cancers in their patients developed on the left side, compared with 28% on the right side ($P < .03$). This is a significant difference to note, especially considering that clear-glass and dark-tinted windows in cars only transmit 62% and 11.4% of ultraviolet (UV) A, respectively.

However, it is important to discuss a likely generational gap that exists between the patients they surveyed and current young drivers. The average patient age in the survey was 67.5 years, which means the majority of patients started their main driving years in the early 1960s. This is important since only in 1968, with the production of the American Motors Corporation's Ambassador, was air conditioning made standard in automobiles.² In 1969, only 54% of domestic automobiles were equipped with air conditioning.³ In addition, 42.4% of adults were smokers in 1965, compared to 20.6% in 2008.⁴ Considering both of these factors, it is likely that at least some of the patients surveyed drove with the windows down far more often than current drivers do, resulting in more exposure to UV radiation, particularly UVB, which is blocked by window

glass. We thought this generational gap also deserved comment, as it may further have contributed to the effects of driving on UV light exposure and the left-sided dominance of actinic lesions in this US driving population.

Benjamin C. Garden, BA, and Patricia L. Myskowski, MD

Dermatology Service, Department of Medicine, Memorial Sloan-Kettering Cancer Center, New York, NY

Funding sources: None.

Conflicts of interest: None declared.

Correspondence to: Patricia L. Myskowski, MD, Memorial Sloan-Kettering Cancer Center, 160 E 53 St, New York, NY 10022

E-mail: myskowsp@MSKCC.org

REFERENCES

1. Kim DP, Chabra I, Chabra P, Jones EC. Sunscreen use while driving. *J Am Acad Dermatol* 2013;68:952-6.
2. Ward's 1969 automotive yearbook. 31st ed. Detroit, MI: Powers and Company, Inc; 1969. p. 116.
3. Constable G, Somerville B. A century of innovation: twenty engineering achievements that transformed our lives. Washington, DC: Joseph Henry Press; 2003. p. 119.
4. Centers for Disease Control and Prevention. Smoking-attributable mortality, years of potential life lost, and productivity losses—United States, 2000-2004. *MMWR Morb Mortal Wkly Rep* 2008;57:1226-8.

<http://dx.doi.org/10.1016/j.jaad.2013.09.059>

High survival rate of harlequin ichthyosis in Japan

To the Editor: Harlequin ichthyosis (HI) (OMIM 242500) is one of the most severe genetic skin disorders, and its clinical features at birth include severe ectropion, eclabium, flattening of the ears, and large, thick platelike scales over the entire body. Neonatal death is common.

We read with great interest the recent commentary by Milstone and Choate¹ on the importance of intensive neonatal care in improving HI outcomes. In their commentary, the authors conducted a critical examination of the intrinsic and extrinsic factors contributing to the prognosis of HI and they asserted that there are many effective measures for improving the outcomes of babies with HI.¹ For example, respiratory failure frequently contributes to neonatal death in HI²; thus, they noted that perhaps intubation should have been done in more cases.¹ A multicenter retrospective questionnaire-based survey in England, Sweden, the United States, Iran,